

An Act

ENROLLED SENATE
BILL NO. 1447

By: Gollihare, Bullard, and
Burns of the Senate

and

Marti of the House

An Act relating to the Oklahoma Employees Insurance Plan; defining terms; requiring certain provisions to be included in certain contract; prohibiting pharmacy benefits manager (PBM) from changing drug placement on a formulary without certain written authorization; prohibiting PBM from prohibiting certain guarantees of set pricing; prohibiting PBM from including certain plan designs without certain impact analysis and certain written approval of certain provisions by state plan; prohibiting PBM from establishing certain less favorable network terms; prohibiting establishment of certain gag clauses by certain PBM; prohibiting establishment of certain contract provisions that create certain conditions that exclude certain pharmacies without certain justification; requiring PBM to disclose certain affiliates or subcontractors prior to awarding of certain contract; requiring certain rebates to be passed through to state plan; construing provisions; establishing certain determination of certain guarantees; requiring PBM to maintain certain System and Organization Controls (SOC) Type 2 certification; requiring PBM to provide certain written impact analysis prior to approval of changes to certain contract provisions; requiring certain impact analysis to include certain provisions; requiring PBM to submit certain report including certain claim-level reporting data; requiring certain report to include certain provisions; allowing certain contract proposal to be scored based on certain provisions; providing for codification; and providing an effective date.

SUBJECT: Oklahoma Employees Insurance Plan

BE IT ENACTED BY THE PEOPLE OF THE STATE OF OKLAHOMA:

SECTION 1. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 1330.1 of Title 74, unless there is created a duplication in numbering, reads as follows:

As used in this act:

1. "Affiliate" means any person or entity that controls, is controlled by, or is under common control with another person or entity;

2. "Affiliated pharmacy" means any pharmacy, mail pharmacy, specialty pharmacy, or pharmacy services provider that is owned, operated, managed, or controlled by a pharmacy benefits manager (PBM) or that otherwise shares profits or losses with the PBM or an affiliate through common ownership, management, or contractual arrangement;

3. "Control" means the possession of the power to direct or cause the direction of management and policies of a person or entity, whether through ownership of voting securities, by contract, or otherwise. Control shall be established if a person or entity owns, controls, or holds with power to vote, ten percent (10%) or more of the voting securities or other ownership interest of the other person or entity;

4. "Operation data" means all data and records generated, received, maintained, or derived in connection with the provision of PBM services to a state plan, including claims data, pricing files, payment and remittance data, adjudication edits, rejection and reversal data, provider network files, pharmacy contracts and amendments, audit records, formulary files, utilization management outputs, communications affecting benefits, and any reports derived from such data;

5. "Rebate" means any consideration or thing of value paid, credited, earned, received, retained, or recovered from a pharmaceutical manufacturer by a pharmacy benefits manager in connection with the purchase, dispensing, coverage, placement, administration, or utilization of a prescription drug or other covered product for a person covered under the state plan, regardless of how such consideration is characterized. Rebate includes, but is not limited to, base rebates, formulary rebates, incentive rebates, credits, price protection payments, market-share incentives, promotional allowances, commissions, educational grants, market-share utilization payments, drug pull-through programs, implementation allowances, clinical detailing payments, rebate submission fees, administrative fees, management fees, rebate contracting fees, contract aggregation payments, and any other similar payment or remuneration. Rebate does not include pharmacy purchase discounts and related service fees attributable to or based on the purchase of drugs by specialty drug pharmacies;

6. "Remuneration" means anything of value, whether in cash or in kind, paid, transferred, or provided to or for the benefit of a PBM, or any affiliate or subcontractor of such PBM, in connection with a prescription drug product or pharmacy benefit including, but not limited to, rebates, discounts, price concessions, administrative fees, services fees, data fees, network access fees, market share fees, formulary placement fees, inflationary or price protection payments, coupon or co-pay offset payments, dispensing or transaction fees, reconciliation payments, and any other payments in cash or credit regardless of characterization;

7. "State plan" means the self-insured plan provided pursuant to the Oklahoma Employees Insurance Plan;

8. "Subcontractor" means any person or entity, other than an employee of the PBM that performs, delegates, supports, or administers any PBM function including claims processing, network administration, rebate contracting or aggregation, formulary or utilization management, mail or specialty pharmacy services, auditing, pricing services, or data hosting, analytics, reporting, or transmission whether under a written or oral agreement;

9. "Total net cost" means, for a stated measurement period, the sum of all amounts paid by the state plan for covered prescription

drugs including, but not limited to, all ingredient cost amounts, dispensing fees, and any other amounts billed to the state plan and all amounts paid by covered members for covered prescription drugs, including co-payments, coinsurance, and deductible amounts, minus all rebates and remuneration required to be passed through to the state plan. Total net cost shall be calculated without regard to the gross rebate volume or the list price of the drug; and

10. "Utilization management" means the same as defined in Section 2562 of Title 63 of the Oklahoma Statutes.

SECTION 2. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 1330.2 of Title 74, unless there is created a duplication in numbering, reads as follows:

A. Any pharmacy benefits manager (PBM) contract with a state plan shall:

1. Declare state plan ownership of all plan-specific operational data including, but not limited to, claims, pricing, rebates, formulary, utilization management outputs, audit data, and any such related reporting maintained by the PBM or any affiliate or subcontractor of such PBM;

2. Provide that the state plan have broad audit rights and shall determine designated auditors over any PBM and any affiliates or subcontractors of such PBM to verify compliance with contract provisions;

3. Establish network adequacy including rural access and compliance with any applicable pharmacy choice and access rules established by contract;

4. Provide that the state plan have exclusive authority over formulary selection, tiering, preferred status, exclusions, specialty designation, and utilization management of any provisions of such contract. Nothing in this paragraph shall be construed to prohibit the PBM from providing recommendations to the state plan for formulary selection, tiering, preferred status, exclusions, specialty designation, or utilization management; and

5. Offer for inclusion in such contract, at the election of the state plan, one or more set pricing guarantee methodologies for covered outpatient drugs including, but not limited to, National Average Drug Acquisition Cost (NADAC) plus a dispensing fee, maximum allowable cost (MAC) pricing, or any objectively verifiable benchmark approved by the state plan.

B. No PBM shall:

1. Modify criteria, add restrictions, or change placement of drugs on a formulary without written authorization by the state plan to make such changes;

2. Restrict or prohibit the state plan from requiring or negotiating set pricing guarantees;

3. Include any plan design or incentive to steer any members of the state plan to a pharmacy affiliated with such PBM unless such plan design or incentive is authorized in writing by the state plan after submission of an impact analysis established pursuant to subsection A of Section 3 of this act;

4. Establish network terms or reimbursement methodologies that are less favorable to nonaffiliated pharmacies than to affiliated pharmacies;

5. Establish gag clauses limiting the state plan's ability to share contract terms, performance metrics, audit results, or cost data with oversight entities, auditors, actuaries, or counsel. Any contract terms shared pursuant to this paragraph shall be subject to reasonable confidentiality; and

6. Establish contract provisions that create reimbursement or network conditions that exclude specific pharmacies from the network without documented and state plan-approved access and cost justifications.

C. The PBM shall be required to disclose any affiliates or subcontractors of such PBM to the state plan prior to the awarding of a contract and shall disclose any additional affiliate or subcontractor at no fewer than thirty (30) days after such affiliate or subcontractor is associated with the PBM.

D. All rebates shall be passed through to the state plan and shall be subject to audit by the state plan, including inspection of manufacturer rebate contracts without redaction. Nothing in this subsection shall be construed to require the pass-through of any amounts authorized as fixed and separately stated administrative fees to the state plan.

E. Any guarantees established by the contract between a PBM and a state plan shall be determined by total net cost and access outcomes and shall not be determined by rebate volume. Such performance guarantees shall include clear remedies in the form of credits payable by the plan for failure to comply with the provisions of the contract.

F. A PBM shall:

1. Maintain System and Organization Controls (SOC) Type 2 certification for the duration of the contract with the state plan; and

2. Provide to the state plan, all plan-specific operational data and reporting in the form and medium prescribed by the plan within the time frame established by the contract, or if not specified, within five (5) business days of the state plan's request for then existing data, and within thirty (30) days after the close of each month for monthly files and reports. Such PBM shall not withhold or delay such data or charge any amount except for the fixed administrative fee for extraction, access, interfaces, licensing, or delivery, including any fees imposed by affiliates or subcontractors of such PBM, and shall remain responsible for compliance even if such data is held or processed by such affiliate or subcontractor.

SECTION 3. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 1330.3 of Title 74, unless there is created a duplication in numbering, reads as follows:

A. The pharmacy benefits manager (PBM) shall provide a written impact analysis to the state plan prior to the state plan approving any change to contract provisions, formularies, utilization management criteria, network design, reimbursement methodologies, or

any plan design or incentive pursuant to paragraph 3 of subsection B of Section 2 of this act. No change shall be implemented unless the state plan authorizes in writing such change. Such impact analysis shall include at minimum:

1. The total number of members impacted by each proposed change;
2. The total net plan cost impact of each proposed change;
3. The member out-of-pocket impact of each proposed change;
4. The state plan cost impact of each proposed change; and
5. The pharmacy access impact of each proposed change, including rural access impacts.

B. The PBM shall submit an electronic report in a form and medium prescribed by the state plan that shall include for each prescription drug claim itemized, claim-level reporting data sufficient to validate the following:

1. The date of dispensing;
2. The National Drug Code (NDC);
3. The drug quantity;
4. The number of days' supply;
5. The dispensing pharmacy identifier in the form of the National Provider Identifier (NPI) and the National Council for Prescription Drug Programs (NCPDP) Number;
6. If the dispensing pharmacy is affiliated with the PBM;
7. The final total payment received by the dispensing pharmacy from all sources;
8. The ingredient cost paid to the dispensing pharmacy;
9. The final dispensing fee paid to the dispensing pharmacy;

10. Any post-adjudication fees, withholds, recoupments, incentives, or other adjustments applied to the claim;

11. The total gross amount billed to the state plan for the claim;

12. The total gross amount billed to the state plan for the drug's ingredient cost;

13. The total gross amount billed to the state plan for the dispensing fee;

14. The total net amount billed to the state plan for the claim;

15. The total net amount billed to the state plan for the drug's ingredient cost;

16. The total net amount billed to the state plan for the dispensing fee; and

17. All other amounts charged in connection with the claim.

C. Any contract term in violation of the provisions of this act shall be voidable and severable at the election of the state plan. Any violation of the provisions of this act may constitute a material breach and may be grounds for termination for cause, in addition to any other remedies available under contract or law.

D. Any request for proposal for PBM services for the state plan shall include:

1. Bidder certification of compliance;

2. A compliance narrative; and

3. Sample contract language demonstrating compliance with the provisions of this act.

SECTION 4. NEW LAW A new section of law to be codified in the Oklahoma Statutes as Section 1330.4 of Title 74, unless there is created a duplication in numbering, reads as follows:

In evaluating proposals for a contract between a pharmacy benefits manager (PBM) and the state plan, the state plan may assign favorable scoring or weighting to proposals based on factors in addition to price including, but not limited to:

1. Accessibility and in-state presence, including the ability of the PBM to provide regular in-person meetings with state officials and the positive economic impact of the PBM's operations within this state, including employment, investment, and other economic activity within this state;

2. Compliance history, including the PBM's record of regulatory actions, legal judgments, settlements, or enforcement actions involving a state, self-insured plan, or governmental entity relating to pharmacy reimbursement, claims processing, audit practices, or other PBM business activities; and

3. Transparent pricing models, including the offering of a flat-fee-per-claim-processed model or a per-member-per-month administrative fee model that minimizes or eliminates spread pricing and other undisclosed revenue streams.

SECTION 5. This act shall become effective November 1, 2026.

Passed the Senate the 26th day of March, 2026.

Presiding Officer of the Senate

Passed the House of Representatives the 28th day of April, 2026.

Presiding Officer of the House
of Representatives

OFFICE OF THE GOVERNOR

Received by the Office of the Governor this _____

day of _____, 20_____, at _____ o'clock _____ M.

By: _____

Approved by the Governor of the State of Oklahoma this _____

day of _____, 20_____, at _____ o'clock _____ M.

Governor of the State of Oklahoma

OFFICE OF THE SECRETARY OF STATE

Received by the Office of the Secretary of State this _____

day of _____, 20_____, at _____ o'clock _____ M.

By: _____